

ICD-0-3
carcinoma, infiltrating lobular, NOS 8520/3
Site: Breast, NOS C50.9
4/19/11
fw

UUID: 810663DD-1718-4FD6-B9C2-77B4C091B3F1
TCGA-AC-A23G-01A-PR
Redacted



Collected
Received:
Reported:

Accession: [redacted]
Acpt / Re: [redacted]

SURGICAL PATHOLOGY REPORT

DIAGNOSIS

A. Right axillary sentinel lymph nodes, excision:
Metastatic adenocarcinoma involving [redacted]; frozen section diagnosis confirmed.

B. Right breast, mastectomy:
Bloom Richardson grade II up to 2.0 cm in maximal diameter as measured on glass slide, upper inner quadrant.
Areas of angiolymphatic invasion suggested.
No in situ component identified.
Histologically unremarkable nipple and negative inked deep surgical margin of resection.
Background of proliferative fibrocystic changes including moderate usual ductal hyperplasia and a single microscopic focus of atypical ductal hyperplasia.
Ancillary studies previously obtained

C. Right axillary contents, regional resection:
Metastatic adenocarcinoma involving [redacted]
The largest lymph node shows a multifocal microgranulomatous reaction associated with refractile foreign material.

Staging sheet #25

Stage: T1cN1MX, stage IIA

Electronic Signature: [redacted]

CLINICAL INFORMATION

CLINICAL HISTORY:
Preoperative Diagnosis: Right breast cancer
Postoperative Diagnosis:
Symptoms/Radiologic Findings:

SPECIMENS:
A. Right axillary sentinel nodes
B. Right breast
C. Right axillary contents

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous P. in-af	Noted	
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed	

[Signature] 4/19/11

SPECIMEN DATA

GROSS DESCRIPTION:
A. The first container A is labeled [redacted] and consists of two lymph nodes measuring 1.2 x 1.0 x 0.5 cm and 1.5 x 1.2 x 0.5 cm. The lymph nodes were examined at the time of surgery and are subsequently entirely submitted in cassettes [redacted] as follows: FSA1--1; FSA2--2.
B. The second container B is labeled [redacted] and right breast and consists of a portion of fibroadipose breast tissue that has been received previously inked and sectioned with attached skin measuring 25.0 x 15.0 x 6.0 cm and weighs 830 grams. The skin surface measures 20.0 x 9.0 cm and is light tan wrinkled. There is an area discolored by blue contrast dye surrounding the nipple measuring 6.0 cm. The nipple is centrally placed and appears grossly normal. The specimen has oriented with a suture at superior. On sectioning there is a firm gray-tan fibrous mass measuring 1.2 x 1.0 x 0.7 cm that is 1.0 cm from the deep margin of resection and is located predominantly within the upper inner quadrant. The surrounding breast tissue reveals fatty fibroadipose breast tissue with fibrous tissue. There are no other lesions grossly identified. Received with the specimen are two cassettes, one to 5; deep margin overlying mass--6; sections from upper inner quadrant--7; lower inner quadrant--8; lower outer quadrant--9; upper outer quadrant--10.
C. The third container C is labeled [redacted] and right axillary contents and consists of a portion of fibroadipose tissue measuring 8.0 x 5.5 x 2.5 cm. Sectioning reveals twelve probable lymph nodes measuring from 0.2 to 1.5 cm. The lymph nodes are entirely submitted in cassette [redacted] as

follows: five probable nodes--1; six probable nodes--2; one lymph node trisected--3 and 4.

INTRA-OPERATIVE CONSULTATION:

FSA: 'Metastatic neoplasm in at least one of the two lymph nodes' on

Addendum: Pathology of the ultrasound-guided core biopsy is positive for invasive lobular carcinoma. Patient needs to be referred to a breast surgeon. Patient was called with results and will followup with [redacted] for the necessary surgical referral.

*** END OF ADDENDUM ***

*** VOICE TO TEXT SYNOPSIS ADDENDUM ***

Invasive lobular carcinoma. Patient needs to be referred to a surgeon.

ADS: (6) Known biopsy - proven malignancy

END OF ADDENDUM ***

FULL RESULT:

Indication: Right breast mass, ultrasound-guided core biopsy.

Vacuum-assisted ultrasound-guided core biopsy: Scans through the right breast at 1:00 again confirm a solid irregular mass. It has shadowing posteriorly. Following local anesthesia, a 10-gauge Cassi device was introduced into the mass under ultrasound guidance, using vacuum assistance, multiple core samples were obtained. Procedure was well tolerated.

Clip placement: Under ultrasound guidance, a biopsy clip was inserted percutaneously into the area of the mass. This was successfully deployed under ultrasound guidance.

Right mammogram, post procedure: A postbiopsy mammogram was obtained. This confirms placement of the biopsy marker clip. It is well within the spiculated mass in the upper inner quadrant.

IMPRESSION:

Irregular solid mass right breast at [redacted] Ultrasound-guided vacuum-assisted core biopsy performed. Clip deployed successfully into the biopsy site. Pathology pending.

PATIENT INFORMATION

PHYSICIAN INFORMATION

SPECIMEN INFORMATION

Collected:

Accession #:

Received:

Acct / Reg #:

Reported:

SURGICAL PATHOLOGY REPORT

*** ADDENDUM REPORT ***

ADDENDUM REPORT NUMBER TWO
BREAST PROGNOSTIC PANEL: Block 2

TEST	RESULT	REFERENCE RANGES
DNA Analysis by Image: DNA Index:	<u>SEE COMMENT</u>	0.9 - 1.1 is Diploid < 0.9 or > 1.1 is Aneuploid

COMMENT: Insufficient tumor volume is present for quantitation of DNA by image analysis.

Her2 Gene Amplification By FISH: NEGATIVE

Average copies per cell:	
Her-2/neu:	1.82
Chromosome 17:	1.67
Ratio:	1.09
	> 2.0 Her2/neu

Case Interpreted by [REDACTED]

Notes:

- Quantitative analysis performed using Chromavision ACIS.
- The Her2 /neu and CEP17 probes are manufactured by Vysis Inc.
- The Her2/neu gene detection by Fluorescence In Situ Hybridization (FISH) was performed using the LSI Her2/neu DNA probe, specific for the Her2/neu gene locus 17q11.2-q12 and the CEP1 DNA probe specific for the alpha satellite DNA sequence at the centromeric region of chromosome 17 (17p11.1-q11.1)
- These tests were developed and performance characteristics determined by the [REDACTED] They have not been cleared or approved by the US Food and Drug Administration. The FDA has determined that such approval or clearance is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Act (CLIA) of 1988 as qualified to perform high complexity clinical laboratory testing.
- Place of Service: Center for Advanced Diagnostics [REDACTED]

Addendum Report Issued By: [REDACTED]

ADDENDUM REPORT NUMBER ONE
BREAST PROGNOSTIC PANEL: (Preliminary test results on block A2).

TEST	RESULT	REFERENCE RANGES
Estrogen Receptor:	<u>POSITIVE</u> (84%)	> 4% is Positive 2-4% is Borderline < 2% is Negative
Progesterone Receptor:	<u>POSITIVE</u> (89%)	> 4% is Positive 2-4% is Borderline < 2% is Negative
KI-67 (MIB1) Proliferation Marker:	<u>BORDERLINE</u> (16%)	> 20% is High 10-20% is Borderline < 10% is Low

PATIENT INFORMATION

SPECIMEN INFORMATION

These results were interpreted by Dr. [REDACTED] An additional addendum report will follow when DNA and Her-2-neu tests are completed.

These tests were developed and performance characteristics determined by the US Food and Drug Administration. The FDA has determined that such approval or clearance is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Act (CLIA) of 1988 as qualified to perform high complexity clinical laboratory testing. They have not been cleared or approved by the US Food and Drug Administration.

Addendum Report Issued By:

DIAGNOSIS

DIAGNOSIS:

10 gauge true cut ultrasound guided needle core biopsies of an irregular 1 x 2 cm mass near the 0100 position within the right breast:
Invasive lobular carcinoma.
Size: Two cores partially involved with largest linear dimension 5 mm as measured from slide A2.
Elston modification of Bloom-Richardson Grade:
Architectural score: 3/3.
Nuclear score: 2/3.
Mitotic score: 1/3.
Total score: 6/9=II.
No in situ carcinoma component present.
No lymphovascular space invasion evident.
No microcalcifications seen.
No other significant proliferative breast disease pattern present.
Paraffin Block A2 submitted for breast cancer prognostic panel with results reported as an addendum (paraffin block A5 suitable if additional material necessary).

Electronic Signature:

CLINICAL INFORMATION

CLINICAL HISTORY:

Preoperative Diagnosis: Core needle biopsy, true cut, ultrasound core biopsy ancillary test if malignant: Estrogen/Progesterone receptors, Her2-neu

Postoperative Diagnosis:

Symptoms/Radiologic Findings: Mammogram, ultrasound, size irregular mass 1 x 2 cm

SPECIMENS:

Right breast

SPECIMEN DATA

GROSS DESCRIPTION:

Received in one container labeled [REDACTED], right breast are multiple (15 to 20) soft pale yellow wispy fragments and cores measuring up to 1.2 cm in greatest dimension. The specimen is entirely submitted in five cassettes labeled [REDACTED] Case color blue.